

Supplementary Appendix

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This appendix has been provided by the authors to give readers additional information about the work.

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Supplementary Methods

Central Laboratory Assessment

Central laboratory assessments were used for calculation of the primary and all other eGFR-based outcomes, and for the calculation of potassium cutoffs (>5.5 and >6.0 mmol/L). In addition, potassium and eGFR were assessed at local laboratories for dosing decisions at each visit. Urine analysis, including urinary albumin-to-creatinine ratio (uACR) and urine protein/creatinine ratio (uPCR), were exclusively measured in a central laboratory during the trial.

Primary Estimand

The attributes of the primary efficacy estimand were as follows:

Attributes for the primary estimand	
Population	Patients with CKD without diabetes*
Treatment	All participants received blinded study intervention (finerenone 10 mg/20 mg or placebo) once daily on top of individually tolerated maximum labeled doses of either an ACEI or an ARB
Variable	eGFR values over time collected every 1 to 4 months during the planned treatment period
Intercurrent events	The 3 most important intercurrent events to consider were: • Death† • Starting chronic dialysis or receiving a kidney transplant# • Premature treatment discontinuation/extended interruption of treatment†
Population-level summary	Total eGFR slope from baseline to 32 months comparing the finerenone treatment group with the placebo group as estimated from a random coefficient two-slope linear spline mixed-effects model

* Further defined in the inclusion and exclusion criteria.

† Handled using a composite strategy. For all scheduled visits after the occurrence of the intercurrent event up to the planned end of treatment, 'failure values' for eGFR were assumed for participants who experienced these ICEs. For further details, see Handling of Missing Data.

‡ Treatment policy strategy was applied: participants continued to be followed up for all planned visits after they discontinued study treatment or had an extended interruption, and any eGFR values collected during these periods were included in the analysis.

ACEi denotes angiotensin-converting enzyme inhibitor, ARB angiotensin-receptor blocker, CKD chronic kidney disease, and eGFR estimated glomerular filtration rate.

Alternative Estimand

To assess the treatment effect on total eGFR slope at 32 months without any influence of terminal intercurrent events (ICE) death, chronic dialysis and kidney transplantation, an alternative efficacy estimand was pre-specified corresponding to the shared-parameter model proposed by Vonesh et al. (2019).¹ This estimand treated these ICEs with a hypothetical strategy, i.e., it estimated the effect if no participant would have experienced death, chronic dialysis or kidney transplantation. The remaining ICE, premature treatment discontinuation/extended interruption of treatment, was also treated with a treatment policy strategy.

Further details on this alternative estimand can be found in Section 5.3.4.1 of the Statistical Analysis Plan Version 3.0.

Main Analytical Approach

The primary analysis of the primary efficacy variable was performed according to the participants' randomized intervention and was based on the full analysis set.

The random coefficient two-slope linear spline mixed-effects model behind the population-level summary is presented below.

For clarity, the model specification included only the treatment-specific intercepts and time variables. In contrast to that, the fixed effects for the stratification factors used in the stratified randomization (i.e., baseline SGLT2 inhibitor use [yes, no] and screening UACR [≤ 1000 mg/g vs > 1000 mg/g]) and their interactions with time are not shown in the model formula as their inclusion is straightforward.

As described in Vonesh et al. (2019),¹ the two-slope linear spline mixed-effects model was formulated as follows:

$$Y_{ij} = (\beta_{0c} + \beta_{1c}t_{ij} + \beta_{2c}\max\{t_{ij} - t^*, 0\})(1 - X_i) + (\beta_{0f} + \beta_{1f}t_{ij} + \beta_{2f}\max\{t_{ij} - t^*, 0\})X_i + b_{0i} + b_{1i}t_{ij} + b_{2i}\max\{t_{ij} - t^*, 0\} + \epsilon_{ij},$$

Where

Y_{ij} denotes the response variable, eGFR, for the i th participant on the j th occasion,
 X_i is a treatment group indicator variable with $X_i=0$ if the i th participant is randomized to the placebo group and $X_i = 1$ if randomized to the finerenone group,

$\beta_k = (\beta_{0k}, \beta_{1k}, \beta_{2k})'$ denotes the vector of regression parameters for placebo ($k = c$) and finerenone ($k = f$),

ϵ_{ij} denote within-participant residuals,

$b_i = (b_{0i}, b_{1i}, b_{2i})'$ denotes the vector of between-participant random effects,

t^* denotes the change point (knot at 0.2464 years, equivalent to Month 3),

$\max\{t_{ij} - t^*, 0\}$ is the truncated line function, and

t_{ij} are the time points (in years) at which eGFR is measured for the i th participant across v_i occasions ($j = 1, 2, \dots, v_i$).

Model assumptions:

- The errors ($\epsilon_{ij}|\mathbf{b}_i$) are conditionally independent normally distributed random variables with mean 0 and within-participant variance σ_e^2
- The participant-specific random effects $\mathbf{b}_i$ are independent multivariate normally distributed with mean vector $\mathbf{0}$ and a 3×3 covariance matrix $\mathbf{\Omega}_k$, where $\mathbf{\Omega}_k$ denotes an unstructured covariance matrix that varies by treatment k .

The interpretations of the regression parameters $\boldsymbol{\beta}_k = (\beta_{0k}, \beta_{1k}, \beta_{2k})'$ in terms of chronic and acute slope for placebo ($k = c$) or finerenone ($k = f$) are as follows:

β_{0k} denotes the intercept for treatment group k ,

β_{1k} is the acute slope up to before (excluding) time t^* (the knot at Month 3) for treatment group k ,

β_{2k} is the directional change in slope starting at time t^* (the knot at Month 3) for treatment group k , and

$\beta_{1k} + \beta_{2k}$ corresponds to the chronic slope for treatment group k .

Finally, under these assumptions, the total slope up to time t describing the rate of change in eGFR from baseline up to a specific time $t > t^*$ is defined for treatment group k as

$$\beta_{3k}(t) = \frac{\{\beta_{1k}t + \beta_{2k}(t - t^*)\}}{t} = \beta_{1k} + \beta_{2k} - \beta_{2k}\left(\frac{t^*}{t}\right).$$

Specifically, this total slope calculation was performed for up to $t = 2.6283$ years (equivalent to 32 months).

The analysis was carried out using maximum likelihood procedures. The model was fitted using the SAS procedure MIXED, with the *containment method* applied to calculate the denominator degrees of freedom for tests of the fixed effects. Linear contrasts were constructed to estimate the acute, chronic, and total eGFR slopes. Estimates of these slopes for both the finerenone and placebo groups were presented along with 95% confidence intervals. For treatment-specific slope estimates, the fixed-effect terms for the stratification factors and their interactions with time were evaluated at a population-average distribution of the randomization strata, using weights proportional to the number of participants in each stratum in the analysis population.

Multiple Testing Procedure

The primary and the key secondary endpoints were tested under a confirmatory hierarchical testing procedure. If the primary efficacy endpoint achieved the significance level of 0.05, the corresponding null hypothesis would be rejected, and the three remaining key secondary endpoints would be tested according to the pre-specified hierarchical sequence given below. Consequently, confirmatory testing of the second, third, and fourth hypotheses required rejecting each preceding null hypothesis. Each hypothesis was assessed at a two-sided significance level of 0.05 in this setting while maintaining the global type I error rate at 0.05.

The pre-defined hierarchical sequence of endpoints was:

1. Mean rate of change of eGFR from baseline to month 32
2. Time to the composite endpoint of sustained eGFR decline of $\geq 57\%$, kidney failure, hospitalization for HF, or CV death
3. Time to the composite endpoint of sustained eGFR decline of $\geq 57\%$ or kidney failure
4. Time to the composite endpoint of hospitalization for HF, or CV death

The widths of the confidence intervals have not been adjusted for multiplicity and the intervals may not be used in place of hypothesis testing for outcomes other than the primary outcome.

Handling of Missing Data

Missing data was defined as data not collected as planned for a given estimand according to the protocol, such as non-analyzable samples or missed visits. It was considered that intermittent missing eGFR values (e.g., due to a single missed visit) would not lead to a biased slope estimation and were missing at random. In contrast, for monotone missing data patterns (i.e., missingness due to loss of eGFR follow-up), sensitivity analyses under different assumptions for the missingness mechanism were performed.

Missing data was also differentiated from data not possible to be measured due to the occurrence of intercurrent events. The composite strategy for the following intercurrent events (i) chronic dialysis or kidney transplantation and (ii) death in the main analytical approach for the primary efficacy outcome (total slope at month 32) was implemented with a resampling imputation procedure, using the following prespecified steps: 1) The earliest occurrence of death, chronic dialysis or kidney transplantation was identified for each participant; 2) For eGFR assessments expected under the primary estimate within the planned treatment period and those that occurred after this time point, the analysis value was treated as missing and imputed using multiple imputation; 3) Imputed values were generated by resampling with replacement from an empirical donor pool of clinically unfavorable eGFR values observed in this study, defined using eGFR derived from the 2009 CKD EPI creatinine equation. This involved identifying potential donors as participants with chronic dialysis or kidney transplantation in the full analysis set. For each potential donor, the last available eGFR measurement prior to the onset of chronic dialysis or kidney transplantation (based on either central or local laboratory) was derived. If the value above was $15 \text{ mL/min/1.73 m}^2$ or less, it was included in the donor pool; otherwise, it was removed. Each missing assessment due to chronic dialysis or kidney transplantation and death, was drawn with replacement from this donor pool; and 4) 50 completed datasets were generated and analyzed using the prespecified primary model, and combined using Rubin's rules.

Sensitivity Analyses for the Primary Endpoint

The potential impact of missing eGFR values after loss of eGFR follow-up in FIND-CKD was investigated using multiple imputation based on treatment discontinuation status and a tipping point analysis. This allowed an assessment of the robustness of the results for the primary efficacy estimand with respect to the impact of missing data. This was implemented using SAS procedures PROC MI and PROC MIANALYZE.

Multiple Imputation with Treatment Discontinuation Indicators

For participants who permanently discontinued study intervention for reasons other than chronic dialysis or kidney transplantation and death, monotone missing eGFR values were imputed using a multiple imputation regression model that conditioned on visit-specific treatment discontinuation indicators.² A participant was considered to be on treatment through the earliest of the last dose date or the start date of an extended treatment interruption. Intermittent gaps were first imputed using Markov chain Monte Carlo (MCMC) multiple imputation draws assuming a multivariate normal distribution for baseline and post-baseline eGFR, thereby producing a monotone dataset. The MCMC model included the fully observed treatment group and randomization stratification factors as predictors, so these variables informed the draws while not themselves being imputed. The remaining monotone missing values were then imputed with each visit being modeled as a function of treatment group, randomization stratification factors, baseline eGFR, the observed values of earlier visits, and the relevant treatment discontinuation flags; an indicator was only included when at least 10 post-discontinuation observations were available at that visit.

Tipping Point Analysis

A delta-adjusted pattern imputation method was used for the tipping point analysis under the Missing Not At Random (MNAR) assumption. In the first step, intermittent missing values were imputed using the MCMC methodology. In the second step, it was assumed that the average of the missing outcomes in the finerenone group deviates by a specified magnitude, referred to as delta, from the values predicted by the multiple imputation model. In contrast, the mean in the control group was assumed to be consistent between missing and observed data. Therefore, missing values for each treatment group were imputed based on observed values within their respective groups based on a multiple imputation model, but only imputed values in the finerenone group were adjusted by adding the respective delta to reflect a worsening of outcomes. The approach follows the second variant discussed in Ratitch, O'Kelly, and Tosiello (2013),³ where a delta adjustment is applied at each visit, starting with the first visit with missing data up to the end of the observation period. In addition to using previously delta-adjusted values as predictors, it works under the assumption that study discontinuation is associated with a worsening process that persists throughout the whole time period after study discontinuation. Multiple shifts were applied until the difference between finerenone and placebo was no longer statistically significant (i.e., the null hypothesis of no difference between finerenone and placebo could no longer be rejected). The shift that resulted in failure to reject the null hypothesis defined the tipping point.

Analysis of eGFR Slopes Using Other eGFR Equations

To account for recent developments in the estimation of GFR excluding race, two alternative race-unadjusted equations for eGFR were used:

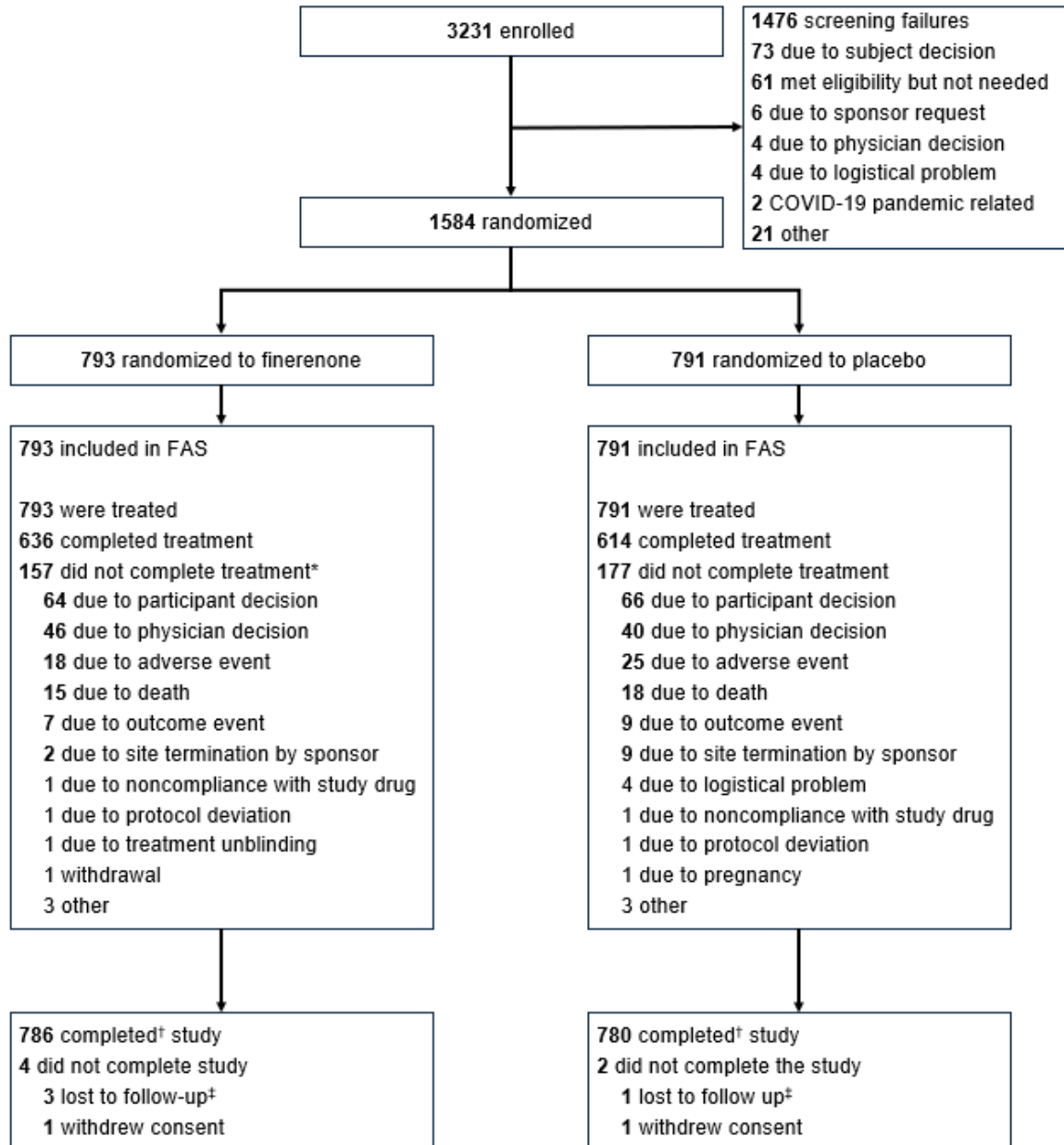
1. CKD-EPI 2021 creatinine (based on serum creatinine, age, and sex)
2. CKD-EPI 2021 creatinine-cystatin C (based on serum creatinine, serum cystatin C, age, and gender)⁴

In addition, the race-adjusted 2012 CKD-EPI creatinine-cystatin C equation (based on serum creatinine, serum cystatin C, age, gender, and race)⁵ was calculated for comparison.

For Japanese participants, creatinine-only CKD-EPI equations (2009 and 2021) were calibrated by multiplying the results by a correction factor of 0.813.⁶ CKD-EPI equations based on creatinine plus cystatin C (2012 and 2021) were modified by a factor of 0.908.⁷

Supplementary Figures

Figure S1. CONSORT Diagram.



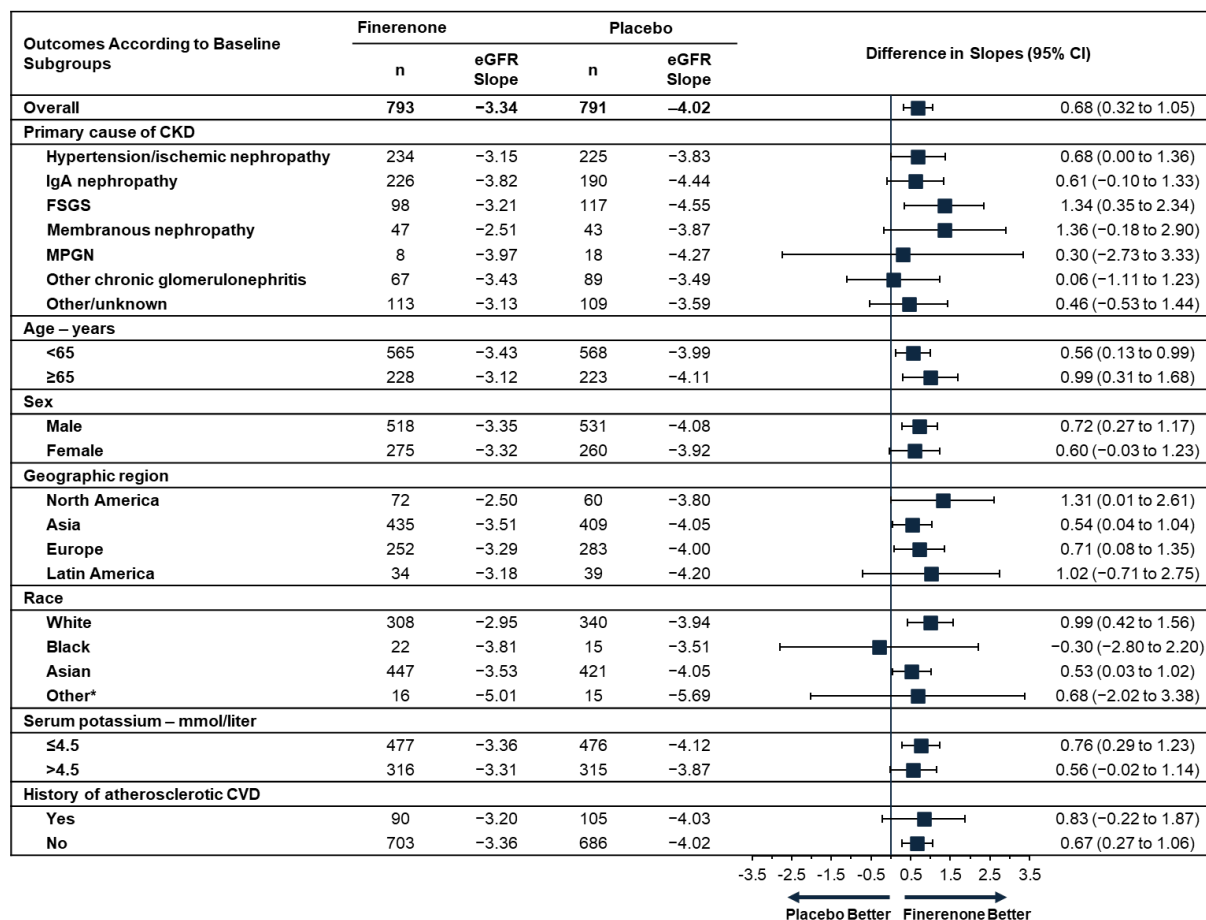
At month 32, 494 (81%) participants in the finerenone group were treated with a daily 20 mg dose and in the placebo group, 535 (88%) received matching placebo treatment.

* Two participants in the finerenone group discontinued treatment for more than one reason. FAS denotes full analysis set.

† Participants were considered to have completed the study if there was contact with the participant after the end-of-study notification or if the subject died.

‡ Excluding participants in Russia (n=3 in the finerenone group, n=9 in the placebo group; all completed the month 32 visit), who were administratively discontinued from the study and all

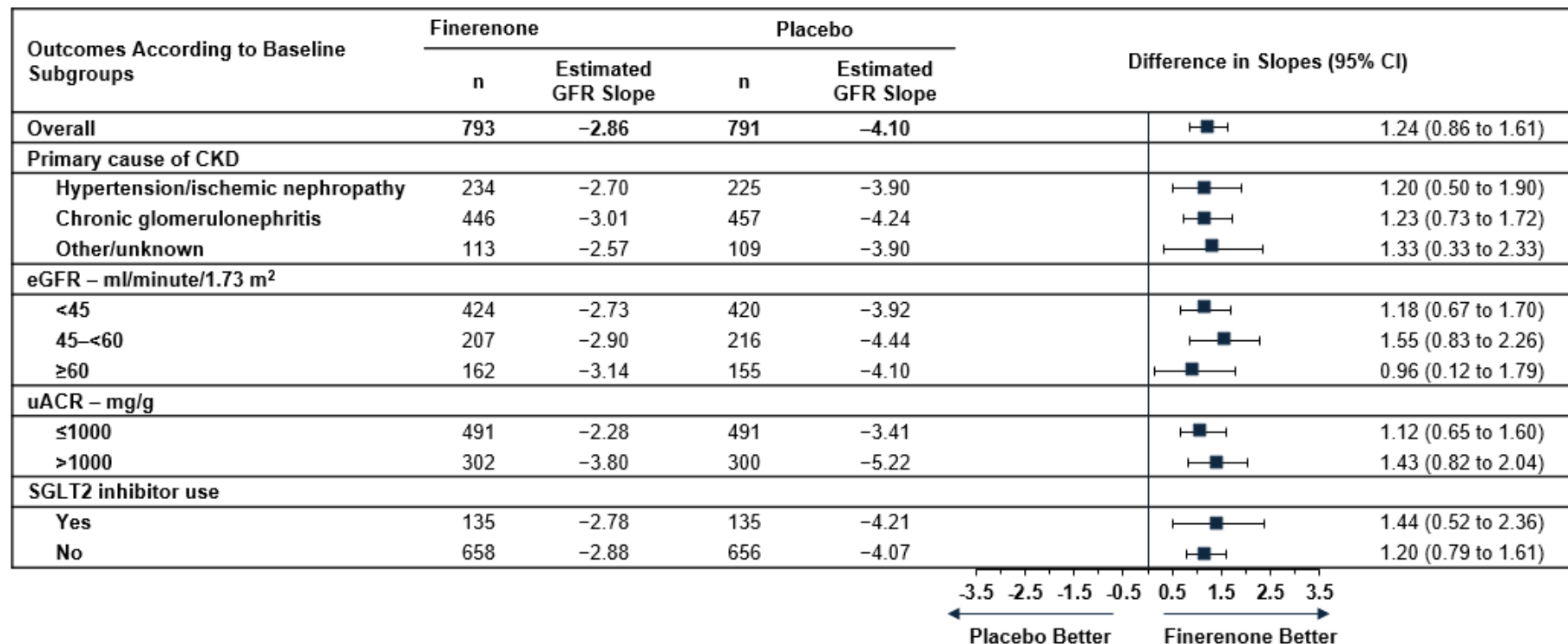
electronic systems closed in Q1/25 to comply with company policy; no further follow-up was possible.

Figure S2. Primary Outcome by Prespecified Subgroups.


Subgroup analysis for mean annual rate of change in eGFR from baseline to month 32 (total slope) according to primary cause of CKD, age at baseline, sex, geographic region, race, baseline serum potassium level, and history of atherosclerotic CVD. Analyzed using a two-slope linear spline mixed-effect model. The square represents the estimate of the total slope difference at 32 months; the lower and upper ends of the whiskers indicate the limits of the 95% confidence interval. CIs have not been adjusted for multiplicity.

* Including American Indian or Alaska native, native Hawaiian or other Pacific islander, not reported or multiple. The multiple-race category included participants who reported belonging to more than one racial group.

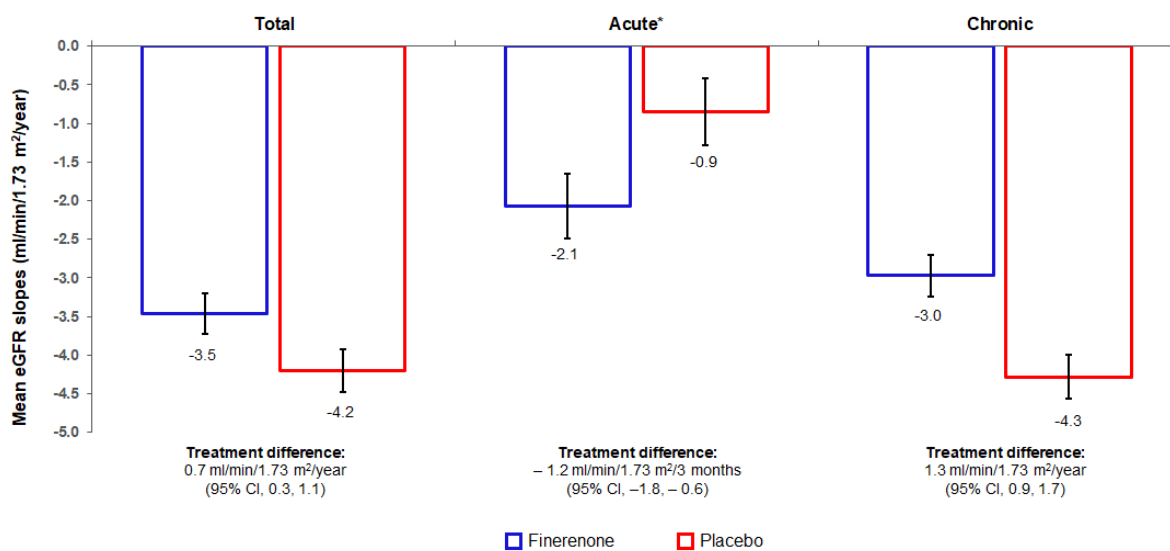
CI denotes confidence interval, CKD chronic kidney disease, CVD cardiovascular disease, eGFR estimated glomerular filtration rate, FSGS focal segmental glomerulosclerosis, IgA immunoglobulin A, and MPGN membranoproliferative glomerulonephritis.

Figure S3. Annual Change in eGFR Chronic Slope in Prespecified Subgroups.

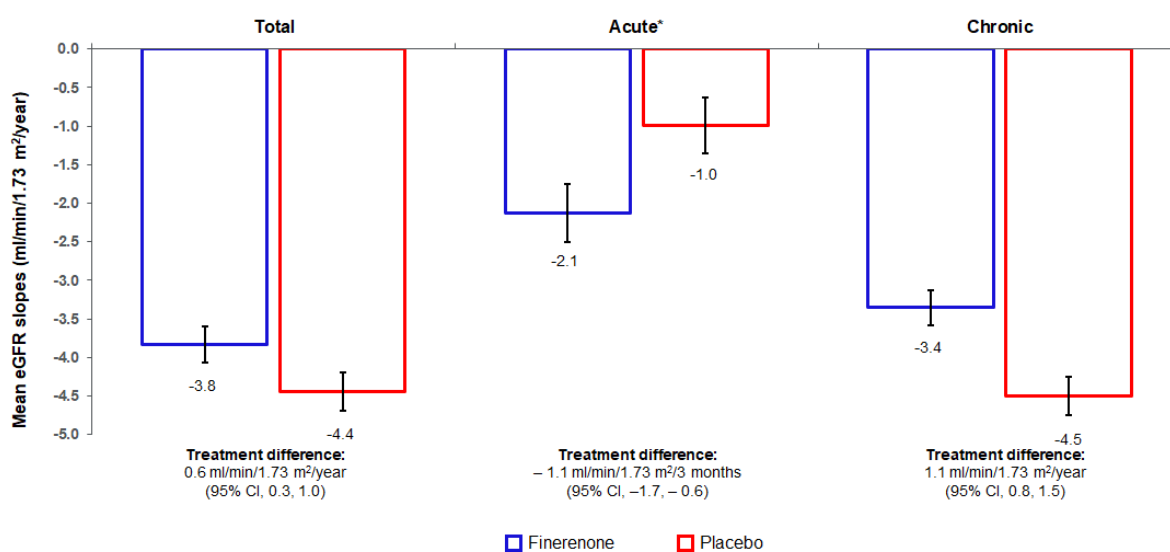
Subgroup analysis for mean annual rate of change in eGFR from month 3 to end of treatment according to primary cause of CKD, eGFR at baseline, albuminuria at screening, and SGLT2i use at baseline. Calculated with the use of a two-slope linear spline mixed-effect model. The square represents the estimate of the mean annual rate of change in eGFR from month 3 until end of treatment (chronic slope); the lower and upper ends of the whiskers indicate the limits of the 95% CI. CIs have not been adjusted for multiplicity. CI denotes confidence interval, CKD chronic kidney disease, eGFR estimated glomerular filtration rate, SGLT2i sodium–glucose cotransporter 2 inhibitor, and uACR urinary albumin-to-creatinine ratio.

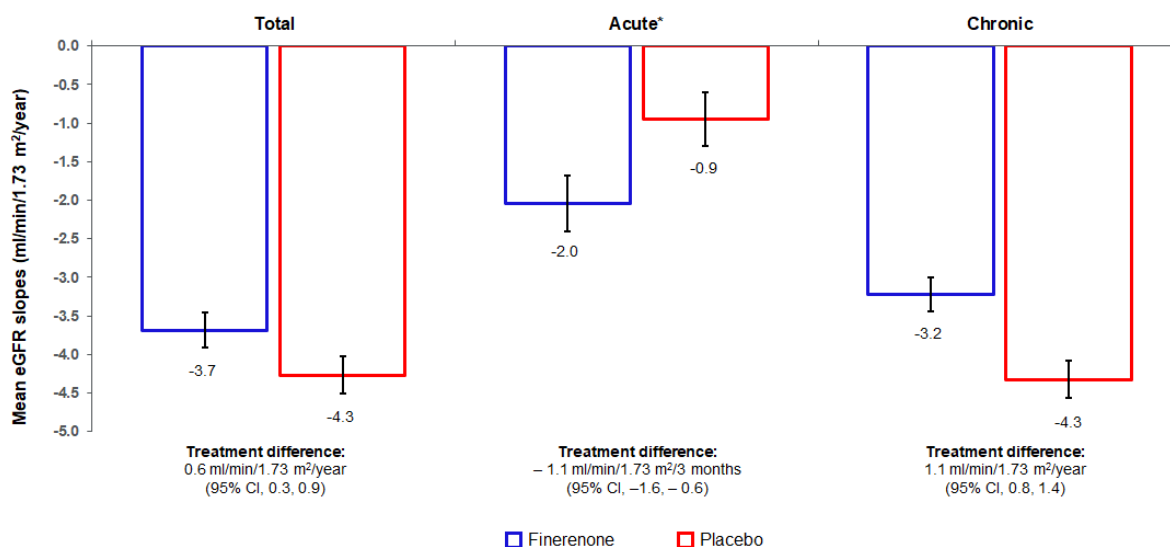
Figure S4. eGFR Slopes Using Different eGFR Equations

A CKD-EPI 2021 creatinine (race unadjusted)

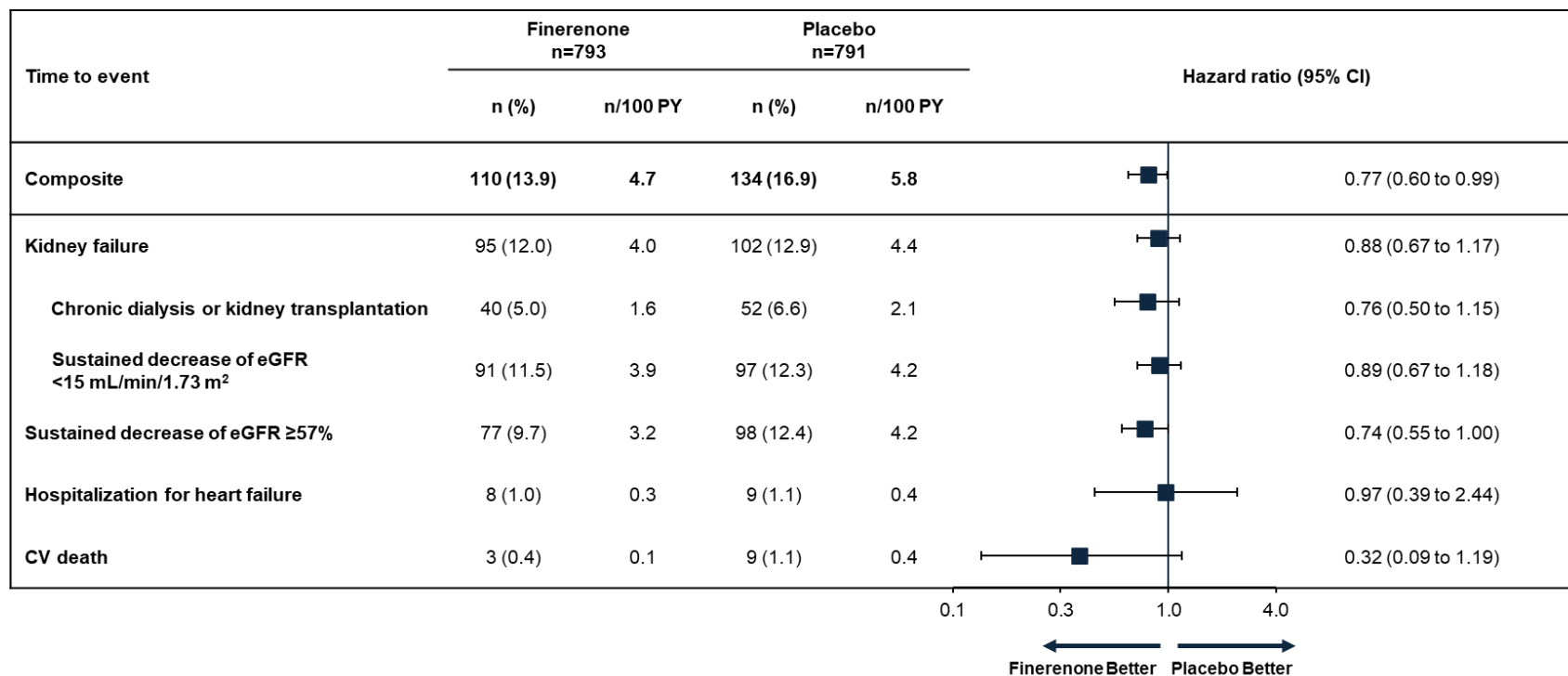


B CKD-EPI 2021 creatinine-cystatin C (race unadjusted)



C CKD-EPI 2012 creatinine-cystatin C (race adjusted)


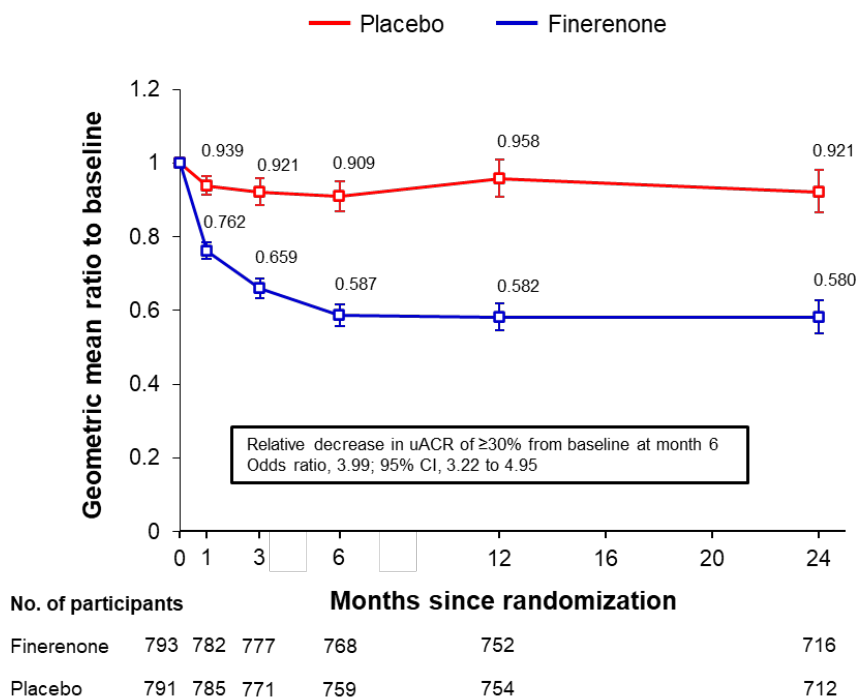
Mean rate of change in eGFR from baseline to month 32 (total slope), from baseline to month 3 (acute slope) and from month 3 until end of treatment (chronic slope) calculated with the use of a two-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization and time difference relative to the change point (month 3); baseline SGLT2 inhibitor use; screening uACR category; and interactions of SGLT2 inhibitor use and uACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. The I bars indicate 95% CI, except where otherwise indicated. CIs have not been adjusted for multiplicity. CI denotes confidence interval, eGFR estimated glomerular filtration rate, SGLT2 sodium–glucose cotransporter 2, and uACR urinary albumin-to-creatinine ratio.

Figure S5. Efficacy for Components of the First Composite Secondary Outcome*

Component and subcomponent tiers were analyzed as separate time-to-event endpoints with endpoint-specific follow-up: eGFR-based components used the eGFR ascertainment window, whereas dialysis or transplant, hospitalization for heart failure, and CV death used standalone follow-up censored at last contact planned treatment end (while-alive for non-CV death) and were not constrained by the eGFR window; counts and rates are not intended to sum to the composite. CIs have not been adjusted for multiplicity, except for the composite outcome, which was adjusted for multiplicity.

* Onset of kidney failure, sustained decrease of eGFR ≥57%, hospitalization for heart failure, or CV death.

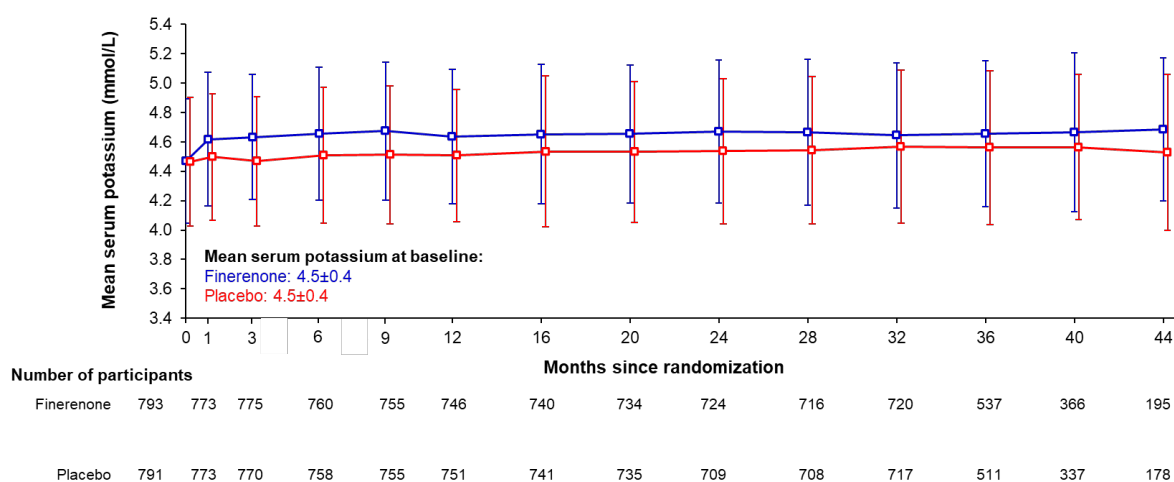
CI denotes confidence interval, CV cardiovascular, eGFR estimated glomerular filtration rate, and PY patient-years

Figure S6. Change from Baseline in uACR

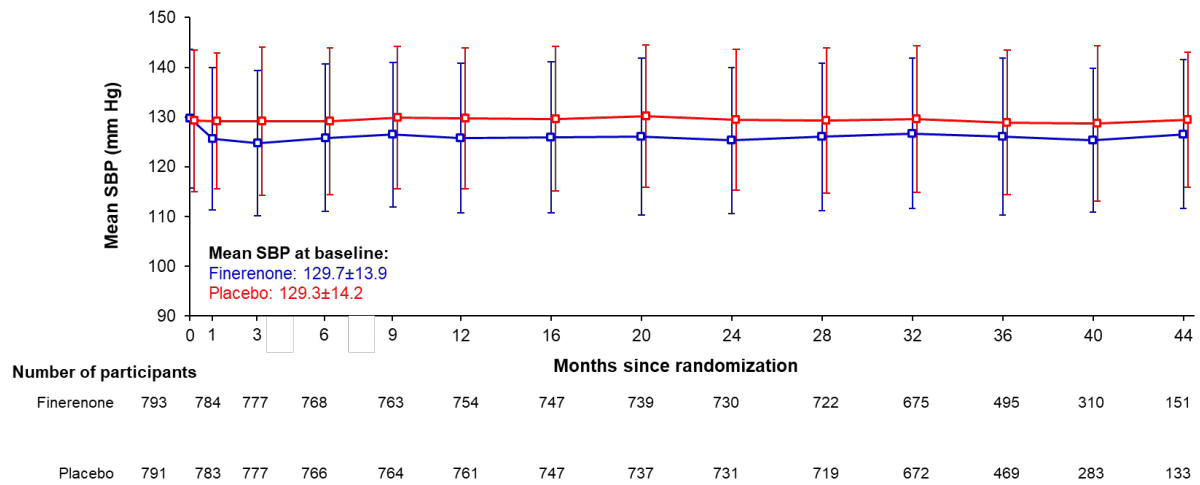
Effects of finerenone and placebo on uACR from baseline to month 24 (full analysis set). Analyzed using a mixed model for repeated measures with fixed effects for the following factors: treatment group, visit, treatment by visit interaction, baseline SGLT2 inhibitor use, and the interaction between SGLT2 inhibitor use and visit, with the log-transformed baseline uACR value as a covariate and an interaction term between the log-transformed baseline value and visit.

I bars indicate the 95% CI. CIs have not been adjusted for multiplicity.

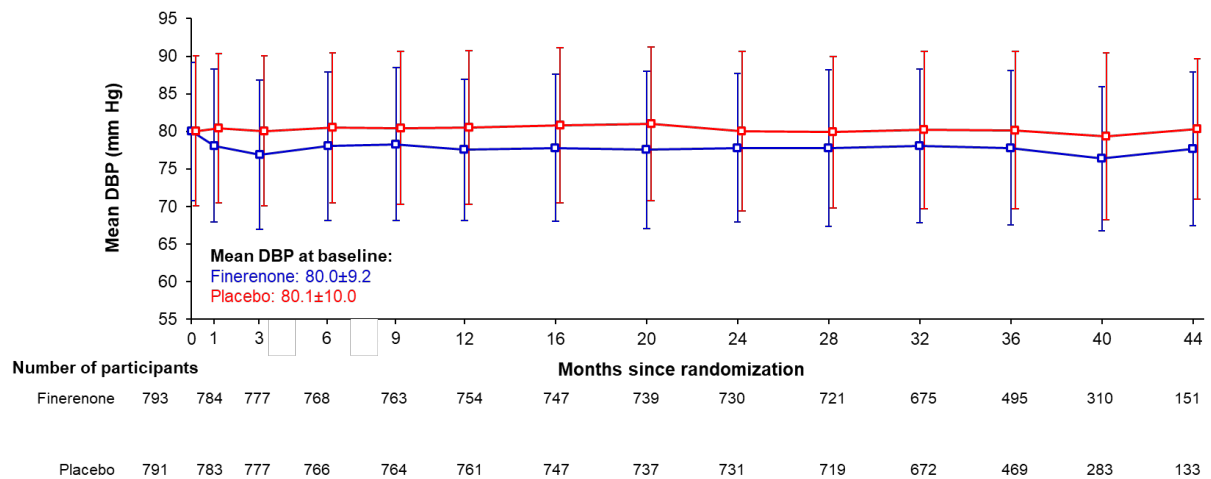
CI denotes confidence interval, SGLT2 sodium–glucose cotransporter 2, and uACR urinary albumin-to-creatinine ratio.

Figure S7. Change from Baseline in Serum Potassium

Effects of finerenone and placebo on serum potassium levels from baseline to 44 months in the safety analysis set. Data from on-treatment and off-treatment periods were combined into a single (pooled) category. Visits may have included remapped data from unscheduled or alternative visits. Means and SDs are shown for each visit.

Figure S8. Change from Baseline in Systolic Blood Pressure

Effects of finerenone and placebo on systolic blood pressure from baseline to 44 months in the safety analysis set. Data from on-treatment and off-treatment periods were combined into a single (pooled) category. Visits may have included remapped data from unscheduled or alternative visits. Means and SDs are shown for each visit.

Figure S9. Change from Baseline in Diastolic Blood Pressure

Effects of finerenone and placebo on diastolic blood pressure from baseline to 44 months in the safety analysis set. Data from on-treatment and off-treatment periods were combined into a single (pooled) category. Visits may have included remapped data from unscheduled or alternative visits. Means and SDs are shown for each visit.

Supplementary Tables

Table S1. Inclusion and Exclusion Criteria.

Inclusion criteria
• Age ≥ 18 years • Chronic kidney disease (eGFR of ≥ 25 to < 60 ml/min/1.73 m² and uACR of ≥ 200 to < 500 mg/g* OR eGFR ≥ 25 to < 90 ml/min/1.73 m² and uACR of ≥ 500 to ≤ 3500 mg/g) • Stable and maximum tolerated labeled dose of an ACEi or ARB for at least 4 weeks prior to screening • Serum potassium level ≤ 4.8 mmol/l • Negative pregnancy test in females of childbearing potential • Capable of providing informed consent
Exclusion criteria
• Type 1 diabetes, type 2 diabetes or HbA1c $\geq 6.5\%$ (48 mmol/mol) • SBP ≥ 160 or DBP ≥ 100 mmHg • Symptomatic HF with reduced ejection fraction with class 1A indication for MRAs • uACR > 3500 mg/g at screening • Autosomal dominant or autosomal recessive polycystic kidney disease • Lupus nephritis or ANCA-associated vasculitis or any other kidney disease requiring immunosuppressive therapy within 6 months prior to screening • Acute kidney injury requiring dialysis within 6 months prior to screening • History of organ transplantation • CV event[†] within 3 months prior to screening or elective percutaneous coronary intervention within 1 month prior to screening • Addison's disease • Hepatic insufficiency classified as Child-Pugh C • Concomitant therapy with other mineralocorticoid receptor antagonists, renin inhibitors, sacubitril/valsartan combination, or potassium-sparing diuretic that cannot be discontinued at least 4 weeks before screening • Concomitant therapy with both ACEi and ARB if one is not discontinued at least 4 weeks before screening • Concomitant therapy with potent CYP3A4 inhibitors or moderate and potent inducers (stopped at least 7 days before randomization)

* Capped at $\sim 10\%$ of the total population to ensure a prespecified ratio for a population at risk of progressive kidney function decline. ACEi denotes angiotensin-converting enzyme inhibitor, ANCA antineutrophilic cytoplasmic autoantibody, ARB angiotensin-receptor blocker, CV cardiovascular, CYP3A4 cytochrome P450,3A4, DBP diastolic blood pressure, eGFR estimated glomerular filtration rate, HbA1c glycated hemoglobin, HF, heart failure, MRA mineralocorticoid receptor antagonist, SBP systolic blood pressure, and uACR urinary albumin-to-creatinine ratio.

† Defined as heart failure decompensation, myocardial infarction, stroke, transient ischemic attack, pulmonary embolism, or elective coronary artery bypass grafting.

Table S2. Medications of Interest: Ongoing at Baseline and Concomitant.

Medication – no. (%)	Ongoing at baseline*		Concomitant†	
	Finerenone (N=793)	Placebo (N=791)	Finerenone (N=793)	Placebo (N=791)
Diuretics	142 (17.9)	148 (18.7)	235 (29.6)	272 (34.4)
β-blocker	201 (25.3)	213 (26.9)	249 (31.4)	284 (35.9)
Calcium channel blocker	398 (50.2)	415 (52.5)	465 (58.6)	520 (65.7)
Statins	416 (52.5)	457 (57.8)	482 (60.8)	531 (67.1)
Platelet aggregation inhibitors (excl heparin)	154 (19.4)	156 (19.7)	202 (25.5)	209 (26.4)

* Medications that were started before or on the day of randomization and continued for at least one day after randomization.

† Medications taken concomitantly with study intervention within and after the treatment period.

Table S3. Representativeness of the Trial.

Category	
Disease under investigation	Chronic kidney disease (CKD)
Special considerations related to:	
Sex and gender	In 2021, the global prevalence and incidence of CKD was higher among females than males in most age groups. ⁸ However, CKD burden in terms of both mortality rates and disability-adjusted life year was higher in males than females at all ages. ⁸
Age	In 2021, global CKD prevalence rates were shown to increase with increasing age, with the highest rates among older adults (≥ 70 years). Incidence rates peaked between 80 and 84 years of age. ⁸ Both incidence rates and disability-adjusted life years for CKD due to hypertension and glomerulonephritis were highest in adults >70 years of age, followed by those aged 50–69. ⁹
Race and ethnic group	Higher risks of kidney disease progression have been reported for Black/African American, Hispanic and Asian individuals compared with White individuals. ¹⁰
Geography	The age-standardized prevalence of CKD was 14.2% globally in 2023. Regions with the highest prevalence include North Africa and the Middle East (18.0%), Central Asia (16.0%), South Asia (15.8%), sub-Saharan Africa (15.6%), Latin America and the Caribbean (15.4%), and high-income areas in the Asia Pacific region (14.2%). Prevalences tended to be lower in the United States (11.7%) and Western Europe (8.5%). ¹¹ The highest prevalence of CKD due to hypertension and glomerulonephritis from 1990 to 2021 was seen in middle sociodemographic index regions. ¹² CKD due to hypertension had the largest incidence increase in East Asia, while glomerulonephritis had the highest incidence increase in South Asia. ¹² The disease burden in terms of CKD etiology varies globally, with hypertension being the leading cause of death in the United States and Canada, Southeast Asia, and Southern sub-Saharan Africa, while glomerulonephritis is a leading cause in Eastern Europe, Central Latin America, and Central/Eastern sub-Saharan Africa. ⁸
Overall representativeness of this trial	The FIND-CKD trial enrolled a geographically diverse population who had CKD without diabetes. The following characteristics were reported by the participants: • Age – reported in years

	• Sex – options included male, female, unknown or undifferentiated • Race – all options that applied could be selected: White, Black or African American, Asian, American Indian or Alaska Native, Native Hawaiian or Other Pacific Islander, and Not reported. Within Asian race, participants could select all options that applied from: Asian Indian, Chinese, Japanese, Korean, Thai and other. There were more male than female participants in this trial (66.2% versus 33.8%) and most had advanced CKD, which may reflect the greater disease burden reported among males globally. The mean age of participants in the trial was 54.7 years, which reflects a middle adulthood population. The proportion of Asian participants was high overall (54.8%), whereas the proportion of Black participants was low (2.3%). The trial included participants from 24 countries worldwide including those in Asia, Europe, Latin America and North America. The most common CKD etiologies present in the FIND-CKD population at baseline were glomerulonephritis (57.0%) and hypertensive/ischemic nephropathy (29.0%); except for type 2 diabetes, these are the two most common CKD etiologies globally.⁹ Most participants were receiving treatment with either an angiotensin-converting enzyme inhibitor (27.5%) or an angiotensin-receptor blocker (72.3%) at baseline, while 17.0% were also receiving an SGLT2 inhibitor. These data suggest that the FIND-CKD population is likely to be representative of individuals with CKD without diabetes who have similar characteristics. The exclusion of participants with symptomatic heart failure with reduced ejection fraction (class 1A indications for steroidal MRAs) may limit the applicability of the findings to broader patient populations.
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CKD denotes chronic kidney disease, MRA mineralocorticoid receptor antagonist, and SGLT2 sodium–glucose cotransporter 2.

Table S4. Alternative Estimand – Shared-Parameter Model – Hypothetical Strategy for Death, Chronic Dialysis, or Kidney Transplantation (Full Analysis Set)

Parameter	Finerenone (N=793)	Placebo (N=791)		
	Estimated GFR slope (95% CI)	Estimated GFR slope (95% CI)	Comparison	Estimate (95% CI)
Total slope at 32 months (mL/min/1.73 m ² /year)	-3.309 (-3.562, -3.055)	-3.936 (-4.204, -3.668)	Difference	0.628 (0.259, 0.996)
Time to death, chronic dialysis or kidney transplantation (finerenone/placebo)			Hazard ratio	0.731 (0.390, 1.072)

A shared-parameter model including a random-coefficient two-slope linear spline mixed-effects model for the longitudinal data (eGFR) and a piecewise-exponential survival model for the endpoint of time to death or the start of chronic dialysis/kidney transplantation.

Longitudinal submodel fixed effects: treatment-specific intercept, acute slope, delta slope at the change point; baseline SGLT2 inhibitor use; screening uACR category; and respective interactions with time. Random effects: intercept, acute slope, delta slope. Survival submodel includes treatment and random intercept, acute and chronic slope effects.

Longitudinal eGFR data collected up to the time of death or start of chronic dialysis/kidney transplantation were included. If no such intercurrent event occurred, the participant was censored at the planned end of treatment in the survival submodel.

eGFR denotes estimated glomerular filtration rate, SGLT2 sodium–glucose cotransporter 2, and uACR urinary albumin-to-creatinine ratio.

Table S5. Number of Participants with Missing eGFR Data During Follow-up.

Event – no. (%)	Finerenone N=793	Placebo N=791	Total N=1584
Up to Month 3*			
Chronic dialysis or kidney transplantation	0	0	0
Death	0	0	0
No chronic dialysis, kidney transplantation or death			
Treatment discontinuation/interruption	20 (2.5)	16 (2.0)	36 (2.3)
Missing eGFR	1 (0.1)	3 (0.4)	4 (0.3)
On treatment	773 (97.5)	775 (98.0)	1548 (97.7)
Missing eGFR	6 (0.8)	10 (1.3)	16 (1.0)
Up to Month 24*			
Chronic dialysis or kidney transplantation	15 (1.9)	13 (1.6)	28 (1.8)
Death	8 (1.0)	13 (1.6)	21 (1.3)
No chronic dialysis, kidney transplantation or death			
Treatment discontinuation/interruption	119 (15.0)	84 (10.6)	203 (12.8)
Missing eGFR	15 (1.9)	16 (2.0)	31 (2.0)
On treatment	652 (82.2)	681 (86.1)	1333 (84.2)
Missing eGFR	3 (0.4)	11 (1.4)	14 (0.9)
Up to Month 28*			
Chronic dialysis or kidney transplantation	17 (2.1)	20 (2.5)	37 (2.3)
Death	9 (1.1)	17 (2.1)	26 (1.6)
No chronic dialysis, kidney transplantation or death			
Treatment discontinuation/interruption	130 (16.4)	96 (12.1)	226 (14.3)
Missing eGFR	18 (2.3)	20 (2.5)	38 (2.4)
On treatment	638 (80.5)	660 (83.4)	1298 (81.9)
Missing eGFR	7 (0.9)	8 (1.0)	15 (0.9)
Up to Month 32*			
Chronic dialysis or kidney transplantation	25 (3.2)	29 (3.7)	54 (3.4)
Death	12 (1.5)	19 (2.4)	31 (2.0)
No chronic dialysis, kidney transplantation or death			
Treatment discontinuation/interruption	135 (17.0)	113 (14.3)	248 (15.7)
Missing eGFR	18 (2.3)	21 (2.7)	39 (2.5)
On treatment	622 (78.4)	632 (79.9)	1254 (79.2)
Missing eGFR	2 (0.3)	7 (0.9)	9 (0.6)
Up to end of treatment*			
Chronic dialysis or transplantation	37 (4.7)	51 (6.4)	88 (5.6)
Death	17 (2.1)	25 (3.2)	42 (2.7)
No chronic dialysis, kidney transplantation or death	159 (20.1)	151 (19.1)	310 (19.6)
Treatment discontinuation/interruption	25 (3.2)	37 (4.7)	62 (3.9)
Missing eGFR	25 (3.2)	37 (4.7)	62 (3.9)
On treatment	581 (73.3)	567 (71.7)	1148 (72.5)
Missing eGFR	7 (0.9)	4 (0.5)	11 (0.7)

* Up to Month X refers to the intercurrent event count. eGFR availability was evaluated based on measurements at or after Month X, or after the considered intercurrent event of premature treatment discontinuation/extended treatment interruption up to Month X, as applicable.

eGFR denotes estimated glomerular filtration rate.

Table S6. Impact of Missing eGFR Values Through Imputation with On-treatment Indicators on the Primary Outcome* (Sensitivity Analysis[†] in the Full Analysis Set).

Slope parameter (ml/min/1.73 m ² /year)	Finerenone (N=793)	Placebo (N=791)	
	Estimated GFR slope (95% CI)	Estimated GFR slope (95% CI)	Difference in slope (95% CI)
Acute slope [‡]	-1.96 (-2.37, -1.55)	0.83 (-1.25, 0.41)	-1.13 (-1.71, -0.54)
Chronic slope	-2.91 (-3.17, -2.65)	-4.14 (-4.41, -3.87)	1.23 (0.86, 1.60)
Total slope	-3.37 (-3.62, -3.12)	-4.06 (-4.33, -3.80)	0.69 (0.33, 1.06)

* The primary efficacy outcome was total eGFR slope, defined as mean annual rate of change in eGFR from baseline to month 32.

† Mean rate of change in eGFR from baseline to month 32 (total slope), from baseline to month 3 (acute slope) and from month 3 until end of treatment (chronic slope) calculated with the use of a two-slope linear spline mixed-effect model. The model included fixed effects for treatment-specific intercept, time from randomization and time difference relative to the change point (month 3); baseline SGLT2 inhibitor use; screening uACR category; and interactions of SGLT2 inhibitor use and uACR with the time components. Random effects are included for the intercept, acute slope, and directional change in slope at the change point. Worst-case imputation after death or chronic dialysis or kidney transplantation, treatment policy with imputation with discontinuation flags. CIs have not been adjusted for multiplicity.

‡ Acute eGFR slopes are presented in ml/min/1.73 m²/3 months.

CI denotes confidence interval, eGFR estimated glomerular filtration rate, SGLT2 sodium–glucose cotransporter 2, and uACR urinary albumin-to-creatinine ratio.

Table S7. Tipping Point Analysis for the Missing eGFR Values on the Primary Outcome* (Sensitivity Analysis[†] in the Full Analysis Set).

eGFR shift under finerenone‡	Finerenone (N=793)	Placebo (N=791)	Difference in slope (95% CI)	P value
	Total slope (95% CI)	Total slope (95% CI)		
0	-3.33 (-3.58, -3.08)	-4.02 (-4.28, -3.76)	0.687 (0.324, 1.049)	0.0002
-5	-3.45 (-3.71, -3.20)	-4.02 (-4.28, -3.76)	0.568 (0.201, 0.935)	0.0024
-10	-3.54 (-3.81, -3.27)	-4.02 (-4.28, -3.76)	0.484 (0.108, 0.859)	0.0116
-15	-3.59 (-3.86, -3.31)	-4.02 (-4.28, -3.76)	0.434 (0.051, 0.817)	0.0262
-20	-3.61 (-3.90, -3.33)	-4.02 (-4.28, -3.76)	0.408 (0.020, 0.795)	0.0391
-25	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.392 (0.002, 0.781)	0.0487
-25.1	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.391 (0.002, 0.781)	0.0489
-25.2	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.391 (0.002, 0.781)	0.0491
-25.3	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.391 (0.001, 0.780)	0.0492
-25.4	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.391 (0.001, 0.780)	0.0494
-25.5	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.390 (0.001, 0.780)	0.0495
-25.6	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.390 (0.001, 0.780)	0.0497
-25.7	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.390 (0.000, 0.779)	0.0498
-25.8	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.390 (0.000, 0.779)	0.0500
-25.9	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.389 (0.000, 0.779)	0.0501
-26	-3.63 (-3.92, -3.34)	-4.02 (-4.28, -3.76)	0.389 (0.000, 0.779)	0.0503

* Mean rate of change in eGFR from baseline to month 32 (total slope) calculated with the use of a two-slope linear spline mixed-effect model.

† The model included fixed effects for treatment-specific intercept, acute slope, and delta slope at the change point; baseline SGLT2 inhibitor use; screening uACR category; and interactions of SGLT2 inhibitor use and uACR with the time components. Random effects are included for the intercept, acute slope, and the delta slope at the change point. The delta slope measures the slope shift at the change point. A zero value means no slope change, effectively reducing it to a single-slope model. Worst-case imputation after death, chronic dialysis or kidney transplantation, treatment policy with MNAR imputation for discontinuations in the finerenone group. CIs have not been adjusted for multiplicity with this sensitivity analysis, but p-values are displayed in this case to show the tipping point when statistical significance of the primary estimand is lost.

‡ This column shows the value of the delta shift applied to imputed values for the finerenone treatment group in the tipping point analysis. A value of -25.9 means that imputed eGFR values under finerenone would have to be reduced by 25.9 ml/min/1.73 m² in order for the total slope difference at month 32 to lose statistical significance.

CI denotes confidence interval, eGFR estimated glomerular filtration rate, MNAR missing not at random, SGLT2 sodium–glucose cotransporter 2, and uACR urinary albumin-to-creatinine ratio.

Table S8. Adverse Events by Primary System Organ Class (Safety Analysis Set).

Adverse Event — no. (%)	Finerenone (N=793)	Placebo (N=791)
Overall adverse events	542 (68.3)	517 (65.4)
Blood and lymphatic system disorders	24 (3.0)	39 (4.9)
Cardiac disorders	28 (3.5)	40 (5.1)
Congenital, familial and genetic disorders	0	1 (0.1)
Ear and labyrinth disorders	9 (1.1)	9 (1.1)
Endocrine disorders	6 (0.8)	18 (2.3)
Eye disorders	15 (1.9)	26 (3.3)
Gastrointestinal disorders	96 (12.1)	111 (14.0)
General disorders and administration site conditions	75 (9.5)	98 (12.4)
Hepatobiliary disorders	21 (2.6)	26 (3.3)
Immune system disorders	6 (0.8)	1 (0.1)
Infections and infestations	207 (26.1)	212 (26.8)
Injury, poisoning and procedural complications	39 (4.9)	43 (5.4)
Investigations	72 (9.1)	78 (9.9)
Metabolism and nutrition disorders	209 (26.4)	176 (22.3)
Musculoskeletal and connective tissue disorders	105 (13.2)	89 (11.3)
Neoplasms	34 (4.3%)	31 (3.9%)
Nervous system disorders	74 (9.3)	72 (9.1)
Product issues	1 (0.1)	1 (0.1)
Psychiatric disorders	10 (1.3)	25 (3.2)
Renal and urinary disorders	84 (10.6)	117 (14.8)
Reproductive system and breast disorders	13 (1.6)	26 (3.3)
Respiratory, thoracic and mediastinal disorders	63 (7.9)	57 (7.2)
Skin and subcutaneous tissue disorders	43 (5.4)	42 (5.3)
Surgical and medical procedures	6 (0.8)	16 (2.0)
Vascular disorders	69 (8.7)	87 (11.0)

The adverse events reported include those that started or worsened after the first dose of study drug up to 3 days after any temporary or permanent interruption of study drug. System organ classes are presented alphabetically. Participants were counted only once within each primary system organ class.

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